

Approach to the newly diagnosed pregnant patient

Does the patient want the pregnancy?

No or patient unsure

Yes

- Discuss the options around continuing with pregnancy, choosing adoption or termination of pregnancy (TOP). If the patient chooses adoption, refer to social worker.
- Discuss future contraceptive needs ➔ 154.
- Determine gestational age by dates and on examination. If unable to determine gestational age, refer for ultrasound.

Patient requests a TOP.

Patient decides to continue with pregnancy.

Gestation < 20 weeks

Gestation ≥ 20 weeks

- Check the following (avoid delaying TOP referral):
 - Screen for STI: if vaginal discharge, rash, itch, lumps, ulcers ➔ 49.
 - Do a cervical screen if needed ➔ 55.
 - Test for HIV ➔ 110.
- Arrange booking as soon as possible (within 2 weeks) at designated facility according to gestation:

- TOP is not an option.
- Discuss possibility of adoption.

Decide if patient eligible for basic antenatal care:

Ask about previous pregnancies and operations. Has patient had any of:

- Stillborn or newborn that died within first 28 days of life
- ≥ 3 consecutive miscarriages
- Birth weight of previous baby < 2500g or > 4500g
- Hospital admission for gestational hypertension or pre-eclampsia
- Surgery to uterus or cervix (caesarean section, fibroid removal, cone biopsy, cervical stitch for cervical incompetence)

No

Yes

Ask about current pregnancy. Does patient have any of:

- Diagnosed/suspected multiple pregnancy
- Age ≤ 16 or ≥ 37 years
- Rhesus negative with antibodies
- Vaginal bleeding
- Pelvic mass
- Diastolic BP ≥ 90 at booking

No

Yes

Ask about general medical problems. Does patient have any of:

- Diabetes
- Kidney disease
- Epilepsy
- Alcohol/drug use disorder
- Heart disease
- Asthma
- TB
- Hypertension

No

Yes

Patient is eligible for basic antenatal care.

- Continue with routine first antenatal visit ➔ 159.
- If ≥ 5 pregnancies or previous postpartum haemorrhage, arrange hospital delivery.

Patient is *not* eligible for basic antenatal care.

- Complete booking/first antenatal visit at this visit, then refer to next level of care ➔ 159.
- If known hypertension: stop ACE-inhibitors (like enalapril), give instead **methyldopa** 250mg 8 hourly and refer.

Gestation is
≤ 12 weeks and 0 days

- Book an on-demand TOP:
 - If < 9 weeks, refer to nearest facility offering medical outpatient TOP.
 - If 9 - 12 weeks, refer for facility-based TOP.

Gestation is
≥ 12 weeks and 1 day

- Book assessment for TOP as soon as possible (before 20 weeks) at facility offering 2nd trimester TOP

Arrange appointment for patient to return after TOP for counselling and contraception.